

PRODUCT LITERATURE

CHLORITON TABLET 4 MG

Each tablet contains

Chlorpheniramine Maleate 4 mg

Preservative

Sodium Benzoate 0.1% w/w

Product Description

Round, yellow coloured tablet with J11 embossed with score on one side and TPI embossed on other side.

Pharmacodynamics

Mechanism of action/Effect:

Antihistaminic (H₁-receptor): Antihistamines used in the treatment of allergy act by competing with histamine for H₁-receptor sites on effector cells. They thereby prevent, but do not reverse, responses mediated by histamine alone. Antihistamines antagonize, in varying degrees, most of the pharmacological effects of histamine, including urticaria and pruritus. Also, the anticholinergic actions of most antihistamines provide a drying effect on the nasal mucosa.

Pharmacokinetics

Chlorpheniramine is readily absorbed from the GIT after oral administration. Protein binding for Chlorpheniramine is 72%. Onset of action is 15 - 60 minutes after oral administration while time to peak concentration is 2 - 6 hours and time to peak effect is 6 hours after oral administration. It is metabolised in the liver and excreted as metabolites via urine within 24 hours. In children Chlorpheniramine has been found to have shorter elimination half-life values.

Indication:

Anti-histaminic therapy

Recommended Dose:

To be taken 2 or 3 times a day

Adult : 1 - 2 tablets

6 - 12 years : 1/2 - 1 tablet

Geriatric patients may be more sensitive to the effects of the usual adult dosage. May be taken with food, water or milk to lessen gastric irritation. Not recommended for children under 6 years of age.

Route of Administration

Oral

Contraindications:

Except under special circumstances, this medication should not be used when the following medical problems exist:

- Hepatic and renal function impairment.

Risk-benefit should be considered when the following medical problems exist:

-Bladder neck obstruction or symptomatic prostatic hypertrophy or predisposition to urinary retention: anticholinergic effects may precipitate or aggravate urinary retention.

-Angle-closure glaucoma or predisposition to angle-closure glaucoma: anticholinergic mydriatic effect resulting in increased intraocular pressure may precipitate an attack of angle-closure glaucoma

-Open-angle glaucoma: anticholinergic mydriatic effects may cause a slight increase in intra-ocular pressure; glaucoma therapy may need to be adjusted.

Warnings and Precautions:

Chlorpheniramine:

When used for treatment of cough and cold;

(a) Not to be used in children less than 2 years of age

(b) To be used with caution and doctor's/ pharmacist's advice in children 2 to 6 years of age.

Pediatrics: Not recommended in newborn or premature infants. This age group has an increased susceptibility to anticholinergic side effects, such as central nervous system (CNS) excitation, and increased tendency toward convulsions. A paradoxical reaction characterized by hyperexcitability may occur in children taking antihistamines.

Elderly patients: Dizziness, sedation, mental confusion and hypotension may occur more frequently in patients over 60 years of age. If anticholinergic side effects such as dryness of mouth and urinary retention (especially in males) occur and continue or are severe, medication should probably be discontinued. A paradoxical reaction characterized by hyperexcitability may occur in geriatric patients taking antihistamines.

Dental: Prolonged use of this medicine may decrease or inhibit salivary flow, thus contributing to the development of caries, periodontal disease, oral candidiasis and discomfort

Atropine-like anti-cholinergic activity:

Use with caution in patients with history of bronchial asthma, increased intraocular pressure, hyperthyroidism, cardiovascular disease or hypertension.

Special-Risk Patients:

Use with caution in patients with narrow-angle glaucoma, stenosing peptic ulcer, pyloroduodenal obstruction, symptomatic prostatic hypertrophy or bladder-neck obstruction. May cause drowsiness in susceptible individuals. If affected, do not operate any vehicles or machinery. Patients sensitive to one of the antihistamines may be sensitive to others.

Interactions with Other Medicament:

Combinations containing any of the following medications depending on the amount present may also interact with this medication:

-Alcohol or CNS depression or other CNS depression producing medications: concurrent use may potentiate the CNS depressant effects of either these medications or antihistamines. Concurrent use of maprotiline or tricyclic antidepressants may potentiate the anticholinergic effects of either antihistamines or these medications.

-Anticholinergics or other medications with anticholinergic activity: Anticholinergic effects may be potentiated when these medications are used concurrently with antihistamines. Patients should be advised to report occurrence of gastrointestinal problems promptly since paralytic ileus may occur with concurrent therapy.

-Monoamine Oxidase (MAO) inhibitors including furazolidone and procarbazine:

Concurrent use of MAO inhibitors with antihistamines may prolong and intensify the anticholinergic and CNS depressant effects of antihistamines. Concurrent use is not recommended.

-Ototoxic medications: Concurrent use with antihistamines may mask the symptoms of ototoxicity such as tinnitus, dizziness or vertigo.

-Photosensitizing medications: Concurrent use of these medications with antihistamines may cause additive photosensitizing effects.

Pregnancy and Lactation:

Safe use of this drug has not been established. Contraindicated in nursing mothers due to increased risk of anti-histamine side effects in new born and infants. Patients should stop nursing if drug is essential

Side Effects

-Central nervous system: drowsiness, sedation, dizziness, disturbed coordination, fatigue, confusion, restlessness, excitation, nervousness, irritability, insomnia, convulsion, hallucination, vertigo, tinnitus, dilated pupils, labyrinthitis.

-Cardiovascular effect: Hypotension, headache, palpitation, tachycardia, hypertension, ECG changes, overdosage can lead to cardiovascular collapse.

-Ophthalmic effect: Blurred vision.

-Genitourinary effect: Difficult or painful urination.

-Respiratory effect: Thickening of bronchial secretions, tightness in chest, wheezing, nasal stuffiness.

-Dermatological effect: Urticaria, rash, photosensitivity.

-Haematological effect: Blood dyscrasias, agranulocytosis, hemolytic anemia, leucopenia, and thrombocytopenia (although rare has been reported).

-Others: Excessive perspiration, dryness of mouth, nose and throat, hypothermia, anaphylactic shock.

- Oedema Eyelid Aggravated

- Vomiting

- Nausea

-Gastritis

-Angioedema

Symptoms and Treatment of Overdose**Symptoms of overdose:**

-Anticholinergic effect: clumsiness or unsteadiness; severe drowsiness; severe dryness of mouth, nose or throat; flushing or redness of face; shortness of breath or trouble breathing.

-Cardiac arrhythmias: fast or irregular heartbeat.

-CNS depression: severe drowsiness.

-CNS stimulation: hallucinations, seizures, trouble in sleeping.

-Hypotension: feeling faint.

-Somnolence: sleepiness or unusual drowsiness

-Over-dosage in infants and children may cause Anticholinergic and CNS stimulant effects or death. Hypotension may also occur in the elderly at usual doses.

Treatment of overdose

Since there is no specific antidote for overdose with antihistamines, treatment is symptomatic and supportive.

- To decrease absorption: Induction of emesis (syrup of ipecac recommended) however, precaution against aspiration is necessary, especially in infants and children. Gastric lavage (isotonic or 0.45% sodium chloride solution) if

patient is unable to vomit within 3 hours of ingestion.

- To enhance elimination: Saline cathartics (milk of magnesia) are sometimes used.
- Specific treatment: Vasopressors to treat hypotension; however, epinephrine should not be used since it may further lower the blood pressure.
- Precaution against use of stimulants (analeptic agents) because they may cause seizures.

Packing

Blister packs of 10 × 10's / Box

Pharmaceutical Precautions

Store below 30°C in a dry place, protected from direct light.

Keep away from reach of children.

Expiry Date

3 years from date of manufacture

Name and Address of Product Registration Holder / Manufacturer:

TERAPUTICS SDN. BHD. (590500-W)

Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Name and Address of Distributor:

ZONTRON PHARMACEUTICALS SDN BHD
(445695-T)

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Malaysia Drug Registration No:

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